Your Special Needs Co-Parenting Report

Parent completing assessment

Sam Parent

Child

K.

Assessment date

2026-07-30

Needs categories addressed

- Medical Needs
- Educational Needs
- Psychological or Behavioural Needs

Your Special Needs Co-Parenting Report summarises the information you provided about your child's medical, educational, developmental, behavioural, or psychological needs. It is designed to help you identify concerns, understand co-parenting challenges, and prepare practical next steps.

This report reflects the perspective of one parent. It is not a diagnosis, custody evaluation, or professional recommendation.

2. Your Family Situation

Child: K.

Identified needs

Medical Needs

- Medication schedule
- Specialist appointments
- Emergency-care plan

Educational Needs

- IEP or Section 504 services
- Classroom accommodations
- Communication with teachers

Psychological or Behavioural Needs

- ADHD-related supports
- Emotional-regulation strategies
- Counselling or psychotherapy

Professionals currently involved

- IEP or 504 team
- Paediatrician
- Psychiatrist
- Psychologist
- Teacher
- Therapist or counsellor

Primary concerns you reported

- Medical Needs: Managing ADHD medication timing across two homes.
- Educational Needs: IEP follow-through varies between homes.
- Psychological or Behavioural Needs: Emotional-regulation strategies work in one home more than the other.

Decisions that need to be made

- Medical Needs: Give the medication at the same time every day, weekends included.
- Educational Needs: Get both parents on the school email list.
- Psychological or Behavioural Needs: Agree on the same three coping tools in both homes.

Areas of agreement

- Medical Needs: Medication is important and needs to be given daily.
- Educational Needs: Our child benefits from the IEP.
- Psychological or Behavioural Needs: Therapy is helping.

Areas of disagreement

- Medical Needs: Weekend timing.

3. Areas of Need

Medical Needs

Concern as described: Managing ADHD medication timing across two homes.

Duration: 1 to 3 years · Urgency: Needs attention within weeks

Current needs

- Medication schedule
- Specialist appointments
- Emergency-care plan

Professional recommendation summary

Long-acting stimulant 10mg every morning at 7am; blood pressure review every 3 months.

Educational Needs

Concern as described: IEP follow-through varies between homes.

Duration: 3 to 12 months

Current needs

- IEP or Section 504 services
- Classroom accommodations
- Communication with teachers

Professional recommendation summary

Extra time on tests, quiet space, weekly check-in.

Psychological or Behavioural Needs

Concern as described: Emotional-regulation strategies work in one home more than the other.

Current needs

- ADHD-related supports
- Emotional-regulation strategies
- Counselling or psychotherapy

4. Your Co-Parenting Strengths

What is already helping your child today.

- You have professional recommendations in writing, which makes them easier to share consistently.
- Both parents attend key appointments, meetings, or evaluations.
- Multiple qualified professionals are already involved.

5. Co-Parenting Challenges

Medical Needs: Both parents receiving the same information

What you reported

On the medical needs pathway you disagreed that this is currently working (both parents receiving the same information).

Why this may matter to your child

When each parent has different notes, dates, or instructions, decisions are made from incomplete pictures and the child ends up carrying the gap.

Status: Priority Concern

Options to consider

- Give both parents access to the patient portal or school communication system.
- Share appointment or meeting summaries within 24 hours.
- Maintain one current medication or accommodations list.
- Ask the treating professional to send written summaries to both parents.

Medical Needs: Open, calm discussion of this concern

What you reported

On the medical needs pathway you were unsure that this is currently working (open, calm discussion of this concern).

Why this may matter to your child

When conversations are tense, decisions get delayed and children absorb the tension. A structured channel reduces the emotional cost.

Status: Needs Attention

Options to consider

- Use email or a co-parenting app so each message has room to think.
- Try BIFF-style messages - brief, informative, friendly, firm.
- Introduce a written agenda for regular check-ins.
- Consider a parenting facilitator or mediator for the recurring topics.

Medical Needs: Consistent daily routines across homes

What you reported

On the medical needs pathway you disagreed that this is currently working (consistent daily routines across homes).

Why this may matter to your child

Predictable routines are the strongest support for children with special needs; even small mismatches can escalate anxiety, meltdowns, or missed doses.

Status: Priority Concern

Options to consider

- Agree on the smallest routine to align first (bedtime, morning, medication).
- Write down the exact steps, times, and who does what.
- Send a photo of the routine to both homes so the child sees it in both places.
- Review after two weeks and adjust together.

Medical Needs: Following professional recommendations in both homes

What you reported

On the medical needs pathway you were unsure that this is currently working (following professional recommendations in both homes).

Why this may matter to your child

Inconsistent implementation makes it impossible to evaluate whether the plan is working and can undo the child's progress.

Status: Needs Attention

Options to consider

- Save the most recent recommendation as a PDF in a shared folder.
- Turn the recommendation into a short checklist both homes use.
- Confirm changes in writing before adjusting the plan.
- Ask the professional to review implementation at the next visit.

Medical Needs: Sharing updates promptly between homes

What you reported

On the medical needs pathway you disagreed that this is currently working (sharing updates promptly between homes).

Why this may matter to your child

Delayed information means missed follow-up, duplicate work, and children being asked to be the messenger.

Status: Priority Concern

Options to consider

- Agree on a single channel for updates (app, email, group thread).
- Set a 24-hour rule for appointment, medication, or school updates.
- Use a shared calendar for all events tied to this concern.
- Automate school and portal notifications to both parents.

Medical Needs: Supplies traveling reliably between homes

What you reported

On the medical needs pathway you were unsure that this is currently working (supplies traveling reliably between homes).

Why this may matter to your child

Missing medication, adaptive equipment, homework, or comfort items disrupts the plan and can create emergencies.

Status: Needs Attention

Options to consider

- Keep a duplicate supply in each home wherever possible.
- Use a labelled travel bag with a printed checklist.
- Include the child in the packing routine as age-appropriate.
- Photograph what has been packed on transition day.

Medical Needs: Reliable transitions and handoffs

What you reported

On the medical needs pathway you were unsure that this is currently working (reliable transitions and handoffs).

Why this may matter to your child

Rocky transitions are one of the most common sources of stress for children with special needs; predictability at the moment of handoff is worth a lot.

Status: Needs Attention

Options to consider

- Standardise the time and place for handoffs.
- Use a written handoff note covering meds, mood, and any incidents.

- Introduce a neutral exchange location if direct interaction is difficult.
- Build in a short buffer so the child isn't rushed through the transition.

Educational Needs: Both parents receiving the same information

What you reported

On the educational needs pathway you were unsure that this is currently working (both parents receiving the same information).

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Educational Needs: Attending appointments and meetings

What you reported

On the educational needs pathway you disagreed that this is currently working (attending appointments and meetings).

Why this may matter to your child

First-hand attendance reduces mistranslation and helps both parents hear the same clinical or educational language.

Status: Priority Concern

Options to consider

- Alternate lead attendance for routine visits.
- Attend key decisions (evaluations, IEP meetings, medication reviews) together where possible.
- Use a phone or video option when one parent can't be present.
- Debrief in writing within a day of any missed appointment.

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6. Child Impact Summary

Possible effects of the current pattern - not assigning blame.

- Your child may experience different expectations, routines, or instructions in each home, which can be confusing and disruptive.
- Inconsistent implementation makes it harder to evaluate whether current supports are actually working.
- When parents work from different information the child can end up carrying the gap, sometimes acting as the messenger between homes.
- The concern you described is affecting the child's daily life; consistency between homes tends to reduce that day-to-day load.

7. Detailed Findings by Category

Medical Needs

Status: Needs Attention

Understanding across parents on this pathway is needs attention; coordination between homes is needs attention.

Recommended parent actions

- Keep doing what is working; check in monthly to confirm nothing is drifting.

Educational Needs

Status: Needs Attention

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Recommended parent actions

- Keep doing what is working; check in monthly to confirm nothing is drifting.

Psychological or Behavioural Needs

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Recommended parent actions

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8. Parent Alignment Summary

- Disagreement about priorities
- Agreement about the child's needs
- Disagreement about implementation between homes
- Disagreement about information

Parents can share the same goal while disagreeing about the method. Naming the type of gap is more useful than trying to be right.

9. Recommended Co-Parenting Structure

Structured Co-Parenting

Your answers point to some coordination or understanding gaps. A structured approach with written procedures, deadlines, and clearly assigned responsibilities usually reduces friction.

Safeguards to consider

- Written communication only for logistics; keep it short and factual.
- Use BIFF-style messages - brief, informative, friendly, firm.
- Written agendas for regular check-ins so both parents come prepared.
- Designate a neutral point person for scheduling appointments.
- Mediation before any nonemergency change to the plan.

If you experience family violence, coercive control, intimidation, or fear in this co-parenting relationship, please complete the SA CoParents Family Violence & Communication Safety Assessment. Do not accept a joint communication structure that puts you or your child at risk.

This report reflects one parent's own answers to the SA CoParents Special Needs Co-Parenting Assessment. It is not a diagnosis, a custody evaluation, a determination of parental fitness, or legal advice. Where appropriate, parents should consult their child's paediatrician, therapist, school, attorney, or another qualified professional.